Schizophrenia (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

Schizophrenia is a psychotic disorder: a break in how a person perceives, thinks, and relates to reality, with a lasting decline in work, relationships, or self-care. In DSM-5-TR it heads the chapter "Schizophrenia Spectrum and Other Psychotic Disorders" (American Psychiatric Association [APA], 2022).

How common: the median lifetime morbid risk is about 7 per 1,000 people, and incidence is about 1.4 times higher in men (McGrath et al., 2008). Schizophrenia-spectrum onset peaks at about age 20.5, with a median of 25 (Solmi et al., 2022). People with schizophrenia lose an average of 14.5 years of life (Hjorthøj et al., 2017).

Counselor framing: this is a serious medical-psychiatric illness. **Psychiatric and medical care come first**; counseling works alongside them, never instead of them.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis.

- **Symptoms:** two or more of the following for a significant part of a month (less if treated), at least one from the first three: **delusions** (fixed false beliefs not shared by one's culture or faith community), **hallucinations** (most often voices), **disorganized speech**, **grossly disorganized or catatonic behavior**, and **negative symptoms**.
- **Functioning and duration:** a clear drop below the earlier level of functioning, and signs lasting **at least 6 months**, including at least 1 month of active symptoms.
- **Exclusions:** not better explained by schizoaffective or mood disorder, a substance or medication, or a medical condition.
- **Specifiers:** course is specified after one year (first or multiple episodes; acute, partial, or full remission; continuous). *With catatonia* may be added, and severity can be rated 0–4 per symptom dimension.

Three symptom clusters: *positive* (delusions, hallucinations, disorganization); *negative* (affective flattening, alogia, avolition, anhedonia, attentional impairment; Andreasen, 1982); and *cognitive* (attention, memory, processing speed, executive function), which is not a criterion but drives much of the disability (APA, 2022).

Differential diagnosis (APA, 2022):

- **Brief psychotic disorder:** 1 day to under 1 month, then full recovery. **Schizophreniform:** 1 to under 6 months. **Schizoaffective:** mood episodes fill most of the illness, plus 2 weeks or more of psychosis without mood symptoms. **Mood disorder with psychotic features:** psychosis only during mood episodes.
- **Delusional disorder:** delusions for 1 month or more without other psychotic symptoms; functioning often fairly intact.
- **Substance- or medication-induced:** one in four substance-induced psychoses later became schizophrenia, 34% after cannabis (Murrie et al., 2020).
- **Medical causes (general clinical consensus):** delirium, seizures, tumors, autoimmune or infectious illness, thyroid disease, and dementia. New psychosis always needs a medical workup.

Assessment tools: the 30-item **PANSS** rates positive, negative, and general symptoms (Kay et al., 1987). The **BPRS** is a brief clinician rating of overall symptoms (Overall & Gorham, 1962). The **SANS** rates negative symptoms reliably (Andreasen, 1982), and its companion SAPS rates positive symptoms.

Suicide risk: lifetime risk is about 5% (4.9% in one reexamination), highest near illness onset (Palmer et al., 2005). Strong risk factors are young age, male sex, higher education, prior attempts, depression, active hallucinations and delusions, and insight into the illness. The only consistent protective factor is receiving and adhering to effective treatment (Hor & Taylor, 2010). Ask directly about command hallucinations.

When counselors must refer (general clinical consensus): refer for psychiatric and medical evaluation at any new or worsening psychosis. Use emergency services for danger to self or others, commands to harm, or inability to care for oneself. Coordinate care with a signed release; counselors do not manage medication.

3. What works: treatment and the evidence

Bottom line for class: antipsychotic medication is the foundation. CBT for psychosis (CBTp), family intervention, and coordinated specialty care add real but smaller benefits.

Medication (referral notes). Relapse at 1 year was 27% on antipsychotics versus 64% on placebo (number needed to treat 3), with more weight gain, movement problems, and sedation (Leucht et al., 2012). Across 32 antipsychotics, clozapine had the largest effect, and side effects differed more than efficacy (Huhn et al., 2019).

CBTp: how it is used

- **Engage and normalize.** Start from the client's goals and distress, not the diagnosis, and present experiences as understandable under stress (general clinical consensus).
- **Formulate.** Map how unusual experiences, appraisals, emotion, and reasoning biases form and maintain symptoms (Garety et al., 2001).
- **Test beliefs about voices.** Distress depends on beliefs about a voice's power and purpose more than on its content (Chadwick & Birchwood, 1994). In the COMMAND trial (n = 197), full compliance with harmful commands at 18 months was 28% with cognitive therapy versus 46% with usual care (Birchwood et al., 2014).
- **Test delusions gently** through shared evidence review and small experiments, never argument. Build coping and relapse plans (general clinical consensus).

CBTp evidence (honest effect sizes): across 34 trials, the effect on target symptoms was 0.40 but about 0.22 in rigorous trials (Wykes et al., 2008). Another meta-analysis found small effects (0.33 overall, 0.25 positive symptoms) that shrank to 0.15 and 0.08 with masked raters (Jauhar et al., 2014). In 53 trials, CBT added to medication reduced positive symptoms more than usual care (SMD −0.30) or supportive therapy (−0.47) (Bighelli et al., 2018). A small trial (n = 74) found cognitive therapy lowered symptoms in people who had chosen not to take antipsychotics; it does not replace medication (Morrison et al., 2014).

Family intervention and psychoeducation. Across 32 trials, family intervention cut relapse (RR 0.55, NNT 7) and improved medication adherence (Pharoah et al., 2010). In 72 trials, family interventions (OR 0.35), family psychoeducation, and CBT (OR 0.45) gave the most robust relapse protection at 1 year (Bighelli et al., 2021).

Coordinated specialty care (first episode). In RAISE-ETP (n = 404), NAVIGATE team care improved quality of life, symptoms, and work or school involvement, more so with shorter untreated psychosis (Kane et al., 2016). Early intervention services beat usual care on all 13 outcomes across 10 trials (Correll et al., 2018). Longer untreated psychosis is modestly linked to worse outcomes (Marshall et al., 2005), so **early referral matters**.

Rehabilitation: social skills training improved community functioning (d = 0.52) and negative symptoms (d = 0.40) (Kurtz & Mueser, 2008). Supported employment (IPS) led to competitive jobs for 61% versus 23% (Bond et

al., 2008). Cognitive remediation brought small gains in cognition (d = 0.29) and functioning (d = 0.22), best alongside rehabilitation (Vita et al., 2021; Wykes et al., 2011).

4. Religion, spirituality, and Christian integration: what the evidence shows

Faith matters to most patients. Among 115 outpatients, religion gave hope and meaning for 71% but spiritual despair for 14%. It supported treatment adherence for 16% and opposed it for 15% (Mohr et al., 2006). Helpful spiritual coping predicted fewer negative symptoms and better functioning and quality of life 3 years later (Mohr et al., 2011). Nonadherent patients more often saw a conflict between their religion and medication: 31% versus 8% of adherent patients (Borras et al., 2007).

Religious coping. In 47 patients with psychosis, spiritual struggle was linked to more suicidal ideation, and positive religious coping to larger drops in depression and anxiety (Rosmarin et al., 2013). A 35-study review also linked positive coping to better adherence and urged building faith into CBTp assessment and formulation (Westhead & Georgiades, 2025).

Religious delusions. They appeared in 24% of 193 inpatients, who were more severely ill than patients with other delusions (Siddle et al., 2002). Rates vary by time, place, and probably personal religiosity (Cook, 2015). A second study found them no more severe, but these patients adhered less and got less support from their faith communities (Mohr et al., 2010).

Spiritual experience versus psychosis. Members of new religious movements endorsed as many unusual beliefs as deluded inpatients but were far less distressed and preoccupied; Christian and nonreligious controls did not differ (Peters et al., 1999). Weigh conviction, preoccupation, distress, functioning, and whether one's faith community shares the belief, not content alone (Pierre, 2001). Culture shapes voices too: U.S. patients more often reported violent commands, and patients in India and Ghana more often described relationships with their voices (Luhrmann et al., 2015).

Intervention studies (few; none are Christian-specific CBTp):

- **Spiritual assessment (randomized, n = 78):** it was well accepted and improved appointment attendance, with no change in adherence (Huguelet et al., 2011).
- **Culturally informed family therapy (69 families):** CIT-S, which includes religious coping, beat psychoeducation on symptoms. It ran 15 sessions versus 3, which confounds the result (Weisman de Mamani et al., 2014). Religious coping went with fewer sessions attended, and most dropouts left before the spiritual segment began (Gurak et al., 2017).
- **Prayer camp plus medication (Ghana, n = 139):** symptoms were lower than with prayer camp care alone (effect size −0.48) (Ofori-Atta et al., 2018).
- **Faith healers with primary care (Nigeria and Ghana, n = 307):** symptoms improved more than with enhanced usual care (PANSS −15.0) (Gureje et al., 2020).
- **Culturally adapted CBTp (Pakistan, randomized pilot, n = 42):** improved symptoms and insight versus usual care (Habib et al., 2015).
- **Religiously modified CBT overall:** 10 studies across depression, anxiety, and schizophrenia; not yet well established, but acceptable to strongly religious clients (Lim et al., 2014).

Clergy. One-quarter of U.S. adults who ever sought help for a mental disorder went to clergy. Nearly a quarter of those seeing clergy each year had the most seriously impairing disorders, and most saw no other professional (Wang et al., 2003).

Honest summary for teaching:

1. **Medication plus CBTp, family work, and coordinated care is the evidence base.** Faith integration supports engagement and coping; it does not replace them.

2. **Evidence on religious adaptation is thin.** We found **no randomized trial of Christian-integrated CBT for psychosis**. The best data show that pairing faith-based care with psychiatric care helps.
3. **Ethics (general consensus):** integrate faith only with consent and a spiritual assessment, and coordinate with the psychiatrist.

5. Practical Christian-integration notes

These apply Dr. Campbell's **identify, challenge, replace-with-Scripture** method, used **gently, after stabilization, and alongside psychiatric care**. *Clinical* items rest on the research above. *Pastoral* items are theological application, not research findings.

- **Identify (clinical).** Take a spiritual history: is faith a comfort or a struggle, and how does the client view illness and medication (Borras et al., 2007; Huguelet et al., 2011)?
- **Challenge (clinical).** Do **not** dispute a delusion's religious content or the client's faith; that harms the alliance. Challenge distress-driving appraisals instead, such as a voice's claimed power or "medicine means weak faith" (Birchwood et al., 2014). Look beyond the label "religious delusion" (Mohr et al., 2010).
- **Replace (pastoral).** Offer Scriptures of God's care, not judgment: Psalm 34:18, Isaiah 41:10, Romans 8:38–39, and 1 Peter 5:7. Luke, "the beloved physician" (Colossians 4:14), can frame medicine as compatible with faith. Avoid themes of demons, sin, or judgment when delusions involve persecution or guilt.
- **Faith community (clinical).** With a signed release, invite the pastor to support treatment rather than replace it (Gureje et al., 2020; Wang et al., 2003).
- **Ministers:** use this for **referral awareness only**. Ministers cannot diagnose; refer suspected psychosis promptly for psychiatric evaluation.

6. Quick classroom takeaways

- Diagnose by 2 or more symptoms (one being delusions, hallucinations, or disorganized speech), 6 months' duration, and functional decline. Rule out substances, medical causes, and mood disorders.
- **Medication is the foundation.** CBTp gives small but real benefits; family work and early coordinated care cut relapse.
- **Faith is often a resource and sometimes a struggle.** Assess it, support positive coping, and never argue religious content. No Christian-specific CBTp trial exists yet.

Sources note

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